

Utrogestan Capsule 100mg

Description : Utrogestan 100 mg, round, and slightly yellow soft capsules, containing whitish oily suspension.

CONTENT :

Utrogestan contains natural physiological progesterone identical to that secreted by the human ovary. Each capsule contains 100mg natural micronized progesterone.

PHARMACOLOGY :

The mechanism of actions is similar to those of natural progesterone in particular : anti – estrogen, anti-androgen, anti-aldosterone and gestagen.

Oral Route :

Micronised progesterone is absorbed by the gut. The increase in progesterone plasma levels starts within the first hour and peak plasma progesterone levels are observed 1 to 3 hours following administration.

Pharmacokinetic studies conducted in volunteers showed that after simultaneous ingestion of two capsules, plasma progesterone levels increased from a mean of 0.13ng/ml to 4.25ng/ml after 1 hour, to 11.75ng/ml after 2 hours, to 8.37 ng/ml after 4 hours, to 2.00 ng/ml after 6 hours and to 1.64 ng/ml after 8 hours.

In view of the retention time of the hormone in the tissue, it appears necessary, in order to obtain impregnation throughout the 24 hours period, to divide the dose into two separate administrations with a 12-hour interval. Individual variations exist, however the same individual should maintain the same pharmacokinetic characteristics at several month distance, to allow for good individual adaptation of the dose.

Metabolization :

In the plasma, the principal metabolites are 20 α - Hydroxy – 4 α pregnanolone and 5 α dihydroprogesterone. 95% of urinary elimination occurs in the form of glycuconjugated metabolites the principle being 3 α 5 β pregnanediol (pregnandiol). These plasma and urinary metabolites are identical to those found during the physiological secretion of the ovarian corpus luteum.

VAGINAL ROUTE :

Absorption :

Progesterone administered vaginally has an optimal therapeutic effect since it induces rapid high plasma levels of progesterone after one hour.

The optimal plasma concentration of progesterone is reached within 2 to 6 hours after the application and remains at an average concentration of 9.7ng/ml over 24 hours after administration with a posology of 100mg in the morning and in the evening. This means recommended posology therefore induces steady physiological concentrations of

progesterone, similar to those observed during the phase of a normal ovulatory cycle. The weak inter-individual variation of the progesterone levels enables one to anticipate the reactions expected with a standard posology. With doses superior to 200mg daily, the progesterone concentrations are similar to those of the first trimester of pregnancy.

Metabolism : Plasma levels of 5β -pregnanolone do not increase.

Urinary elimination is mainly observed in the form of 3α , 5β -pregnanediol (pregnandiol) as is revealed by its progressive increase concentration (142ng/ml maximal concentration at 6 hours)

INDICATIONS :

Oral route

Utrogestan can be used in all the circumstances in which the secretion of progesterone by the ovaries is insufficient, in particular: in women with premenstrual syndrome, treatment of menstrual irregularities (In women with menstrual abnormalities or secondary amenorrhoea due to normogonadotrophic amenorrhoea (see Section DOSE AND ROUTE OF ADMINISTRATION), as Hormone replacement therapy (adjunctive use with an estrogen in postmenopausal women with an intact uterus) and in women with menace of abortion or prevention of recurrent abortions by documented luteal insufficiency.

Vaginal route

Luteal Support of Assisted Reproductive Technology (ART) cycles and treatment of unexplained threatened miscarriage in women with bleeding in current pregnancy and a history of at least three or more previous miscarriages. Use in women with less than three miscarriages may be warranted in those with reduced chances of future pregnancy.

DOSE AND ROUTE OF ADMINISTRATION :

Oral Route :

The standard daily dosage regimen in women with luteal phase insufficiency 200 to 300 mg of progesterone taken in one or two doses (i.e. 200 mg in the evening before bedtime and another 100 mg in the morning, if needed. In menstrual irregularities due to ovulation disorders or anovulation the treatment is administered over 10 days per menstrual cycle, usually from cycle days 17 to 26 inclusive.

In women with secondary amenorrhoea, the treatment may be given as a single daily dose of 400 mg (4 capsules 100 mg) at bedtime for 10 days. Prior to use for this indication, other causes of secondary amenorrhoea such as outflow obstruction, pregnancy, prolactinoma, thyroid disorders, pituitary and hypothalamic disorders should be excluded.

In women receiving estrogen replacement therapy with intact uterus, the adjunctive use of progesterone at a dose of 200 mg daily at bedtime should be administered for twelve days in the last half of each therapeutic cycle (beginning on day 15 of the cycle and ending on day 26). Withdrawal bleeding may occur in the following week. Alternatively, 100 mg can be given at bedtime from day 1 to day 25 of each therapeutic cycle, withdrawal bleeding being less with this treatment schedule.

Vaginal Route :

In sterility oocyte donation program: The recommended dose of progesterone is 100 mg on day 13 and 14 of the transfer cycle, followed by 100 mg of progesterone in the morning and evening, from days 15 to 25 of the cycle. From day 26, the dose should be increased- in early pregnancy-weekly, from 100 mg progesterone per day up to a maximum of 600 mg of progesterone per day as three divided doses. This dosage should be continued at least until day 60

In luteal phase supplementation during ART (i.e. In Vitro Fertilization), treatment should be started from the evening of the transfer, as 600 mg of progesterone daily dose in three divided doses morning, midday and evening).

In partial luteal phase insufficiency (dysovation): treatment should be given for 10 days per cycle, usually from days 17 to 26 of the cycle at a dosage of 200 to 300 mg of progesterone daily. The treatment must be interrupted, as soon as possible, in case of amenorrhea and diagnosis of pregnancy.

In the treatment of unexplained threatened miscarriage, the usual dose is 400 mg twice a day (morning and night). Treatment should be initiated at the first sign of vaginal bleeding during the first trimester of pregnancy and should continue to at least the 16th week of gestation.

SIDE-EFFECTS :

Oral route : Somnolence or dizziness may occasionally occur in some patients 1-3 hours after ingestion of Utrogestan. In this case, the dose should be reduced or readapted: 2 capsules at bedtime, from the 12th to the 14th day of the cycle, or vaginal administration. Shortening of the cycle or breakthrough bleedings : Take the treatment later in the cycle (For example start the treatment the 19th day of the cycle instead of the 17th day).

Vaginal Route :

No local intolerance (greasy discharge, pruritus or irritation) has been noted during the different clinical studies.

No other general adverse effects have been noted, in particular somnolence or dizziness during clinical studies with the recommended dosages.

CONTRA-INDICATIONS :

- Known allergy or hypersensitivity to progesterone or to any of the excipients.

- Severe hepatic dysfunction.
- Undiagnosed vaginal bleeding
- Known missed abortion or ectopic pregnancy
- Mammary or genital tract carcinoma
- Thromboembolic or thrombophlebitis disorders
- Cerebral haemorrhage
- Porphyria

WARNINGS :

More than half of early spontaneous abortions are due to genetic complications. Moreover, infections manifestations and mechanical disorders can induce premature abortions. Therefore, the only effect of administration of progesterone would be to delay the evacuation of a dead egg (or interruption of a non-progressive pregnancy). The use of progesterone should be restricted to cases of corpus luteum deficiencies. The treatment in the recommended conditions of use is not contraceptive.

PRECAUTIONS OF USE :

Oral Route : It is recommended to use the treatment after mealtimes, preferably at bedtime. Caution should be taken when driving or using machines. Utrogestan may cause drowsiness and /or dizziness in a minority of patients.

Drug Interactions :

Unknown.

Overdosage : No reports of ill effects from over dosage have been reported. If a large over dosage is discovered within 2-3 hours and treatment seems desirable, gastric lavage is recommended. There are no special antidotes and treatment should be symptomatic.

STORAGE : Keep in cool dry place. Protect from light. Store below 30°C. Shelf-life 3 years from the date of manufacture.

PRESENTATION : Capsules 15's per strip. 30's /box.

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